THE MATERNITY METHOD

NCLEX CHEAT SHEET

TORCH Infections

Maternal & Newborn › Infections in Pregnancy

The Big Picture

TORCH infections can **cross the placenta** or pass to baby **during birth**, causing **birth defects**, preterm birth, or loss. Mom may have **mild or no symptoms**, so **screening and prevention** are key. Common newborn clues: **small head, poor growth, hearing/vision problems, rash, big liver/spleen**.

KNOW THE NUMBERS

1st visit	36 wks	12 hrs	4 wks	Never
Screen **syphilis, HIV, hep B**, rubella	Start **herpes** suppression	**HBIG + Hep B** vaccine for baby	No pregnancy after **MMR**	**Live** vaccines during pregnancy

MEMORY HACK

T · O · R · C · H

T	O	R	C	H
Toxoplasmosis	**Other**: syphilis, varicella, parvovirus, HIV, hep B, Zika	**Rubella**	**Cytomegalovirus** (CMV)	**Herpes** simplex

The Infections

Infection	How mom gets it	Risk to baby	Prevent / treat
Toxoplasmosis	**Cat litter**, **raw/undercooked meat**, soil, unwashed produce	Hydrocephalus, **eye damage** (chorioretinitis), brain calcifications	**Someone else changes litter** (or gloves), cook meat well, wash produce, garden gloves
Syphilis	Sexual contact	Stillbirth, **rash on palms/soles**, "snuffles", bone problems	**Penicillin G** (desensitize if allergic); screen early, again at 28 wks + birth if at risk
Varicella	Chickenpox exposure	Limb defects, scarring; **severe** newborn infection near delivery	Vaccine **before** pregnancy; **VZIG** after exposure
Parvovirus B19	"Fifth disease", often from kids	**Fetal anemia**, hydrops	Hand hygiene; monitor baby with ultrasound
HIV	Blood, sex, perinatal	Transmission in pregnancy, birth, breastfeeding	**ART all pregnancy**; IV zidovudine in labor if viral load high/unknown; avoid AROM, FSE, episiotomy; baby gets meds
Hepatitis B	Blood, sex	Chronic liver disease	Baby: **HBIG + vaccine within 12 hrs**; **breastfeeding OK**
Rubella	Respiratory droplets	**Deafness**, **cataracts**, heart defects (PDA), "blueberry muffin" rash	Check titer; **MMR postpartum** if non-immune, **never** during pregnancy
CMV	**Saliva/urine of toddlers**, sex	**Hearing loss** (#1 non-genetic cause), small head, vision problems	**Hand washing** after diapers; don't share cups/utensils with young kids
Herpes (HSV)	Sexual contact	Skin/eye/mouth, brain, or **body-wide** infection (can be fatal)	**Acyclovir/valacyclovir from 36 wks**; **active lesions in labor → cesarean**

Nursing Care

- **Screen** at the first visit (and repeat as needed)
- **Teach prevention** (litter, food safety, hand washing)
- **Standard precautions**; contact precautions for lesions
- Avoid **internal monitors** with HSV/HIV/hep B
- Treat partners for **STIs**

Breastfeeding Rules

- **Hep B:** OK (after baby's HBIG + vaccine)
- **Herpes:** OK unless a **lesion is on the breast**
- **CMV:** generally OK for term babies
- **HIV:** formula removes the risk; breastfeeding only with a provider plan and suppressed viral load
- **Varicella:** pump; avoid contact with lesions

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RED FLAGS · STOP AND CALL THE PROVIDER

Herpes lesions or prodrome (tingling) at labor · **Rash + fever** in pregnancy · Newborn with **small head**, rash, **big liver**, jaundice · Newborn **lethargy**, poor feeding, seizures

MUST-KNOW FOR THE NCLEX

- **Toxo:** no cat litter, no raw meat.
- **Rubella:** MMR **after** birth, never during; avoid pregnancy 4 weeks.
- **Herpes:** active lesions in labor → **cesarean**; suppress from 36 wks.
- **Hep B baby:** HBIG + vaccine within 12 hrs; breastfeeding OK.
- **CMV** = most common congenital infection; #1 non-genetic cause of hearing loss.

QUICK CHECK · TEST YOURSELF

1. A pregnant patient should avoid changing _____ to prevent toxoplasmosis.
2. Congenital rubella classically causes deafness, _____, and heart defects.
3. Active genital herpes lesions at labor → plan a _____.
4. A baby born to an HBsAg+ mom gets _____ + _____ within 12 hours.
5. The drug of choice for syphilis in pregnancy is _____.

Answers: 1. cat litter · 2. cataracts · 3. cesarean birth · 4. HBIG + hepatitis B vaccine · 5. penicillin G

For study use only. Always follow facility protocols, current guidelines, and provider orders.

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